

PATIENT ENGAGEMENT PILLAR · CS595-LOF-2026-001 · MARCH 2026

ScribeGuard

The richest moment of care is the conversation.
Right now, it vanishes.

AI-Powered Ambient Clinical Documentation with OpenMRS
Integration



THE NEED

A Crisis Hiding in Plain Sight

Physicians write SOAP notes from memory hours after the encounter. Critical clinical detail fades, notes grow inconsistent, and the standard of care suffers with every rushed, retroactive entry. Meanwhile, the tools that could help simply don't exist for the platforms most of the world relies on.

2+

Hours/Day Lost

Per physician on note-writing—time taken directly from patient care

50%+

Physician Burnout

Of all U.S. physicians report burnout, with documentation overhead as a leading driver

500+

Facilities with No AI Tools

OpenMRS sites worldwide with zero access to AI-assisted documentation tools



THE NEED · DEEPER CONTEXT

Three Dimensions of the Problem

Documentation Overload

Physicians write SOAP notes from memory, hours after the encounter. Critical clinical details fade, notes become inconsistent, and care quality erodes with every delayed, rushed entry.

Time Extracted From Care

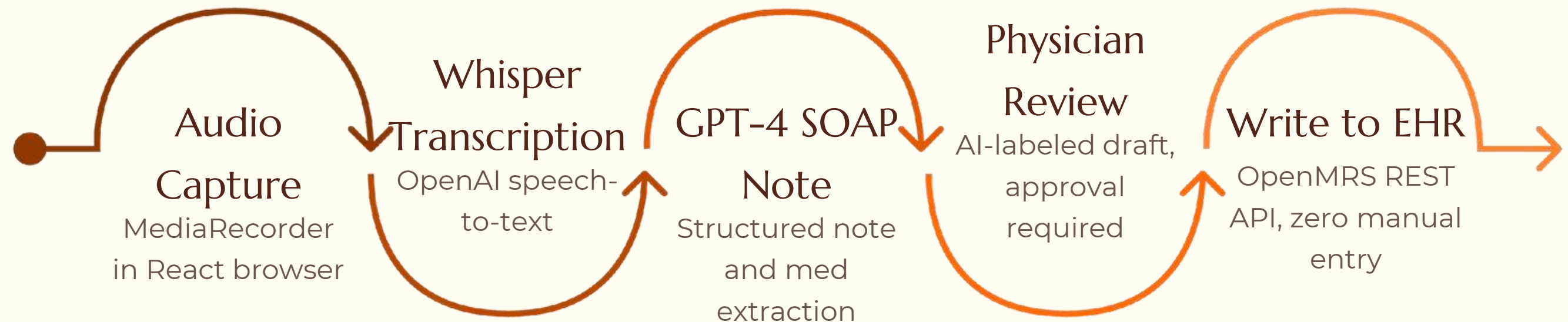
Every minute at the keyboard is a minute not spent with patients. Documentation overhead has become the single largest non-clinical burden in modern medicine.

An Ignored Market Gap

Commercial AI scribes target Epic and Cerner. OpenMRS used by 500+ facilities across low-resource global settings has been left entirely behind.

5-Step AI Pipeline, End-to-End

ScribeGuard captures the clinical conversation in real time and transforms it into a structured, physician-reviewed SOAP note and writes it directly to OpenMRS with no manual re-entry.



No note is ever auto-committed. Physician approval is the final gate!

THE BENEFITS

Real Value. Measurable



2+ Hours Returned Daily

Per physician, per day reclaimed from documentation and given back to direct patient care.



Zero Manual Re-Entry

Physician-approved notes are written directly to OpenMRS via REST API. No typing, no transcription errors.



Physician Always in Control

Every draft is AI-labeled and fully editable. Explicit approval is required; it cannot auto-commit, ever.



Full Audit Trail

Every version stored: transcript → AI draft → edits → final submission, with timestamps throughout.



Higher-Fidelity Records

Notes were generated from the actual conversation and are more complete and clinically accurate.



Open-Source First

The first AI documentation layer for OpenMRS - built for 500+ global facilities left behind by commercial tools.

Where Every Competitor Falls Short

Solution	Open Source	OpenMR S Integ.	AI SOAP Notes	Cost
ScribeGuard ✓	✓	✓	✓	Free
Nuance DAX (Microsoft)	X	X	✓	\$\$\$
Suki AI	X	X	✓	\$\$\$
Abridge	X	X	✓	\$\$\$

Competitors

- X Closed-source
- X Epic/Cerner only
- X Expensive

ScribeGuard

- ✓ Open-source
- ✓ OpenMRS native
- ✓ Free

The solution built for the 500+ facilities competitors ignore.

Know It Before It Bites You — and Our Plan to Stay on Track

Risk Matrix

Risk	Likelihood	Impact	Level
GPT-4 hallucinated clinical content	Medium	High	HIGH
OpenMRS REST API version mismatch	Medium	High	HIGH
Whisper errors in medical terminology	High	Medium	HIGH
Six-week timeline too short for polish	Medium	Medium	MED
Team unfamiliar with FastAPI / OpenMRS	Low	Medium	MED

Mitigation Plan

- AI Hallucinations

Physician-gated review. Every draft labeled "AI-Generated — Pending Review."
- API & Accuracy Failures

REST API validated in Sprint 1. Medical glossary injected into GPT-4 context.
- Timeline & Skill Gaps

Scope narrowed to Layer 1 MVP. MediaRecorder saves a full sprint.